**MERIDIAN CAPITAL PARTNERS**

**Fixed Income Research - Regional Healthcare Services Sector Report**

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**Sector Overview**

The regional acute-care and outpatient services sector has shown steady if unspectacular fundamental performance over the past several quarters. Volumes have recovered to pre-pandemic levels across most service lines, with outpatient and ambulatory surgery centers continuing to take share from traditional inpatient settings as reimbursement and clinical practice both favor lower-acuity, lower-cost care where clinically appropriate.

Labor costs remain the dominant credit story for the sector. Contract labor utilization has declined meaningfully from its post-pandemic peak, but base wage inflation for registered nurses and allied health professionals continues to run above the broader services sector, compressing margins for operators unable to pass costs through via payor rate increases.

Payor mix continues to shift gradually toward managed Medicare and managed Medicaid products, which generally reimburse at lower rates than traditional fee-for-service Medicare and commercial insurance. Operators with scale and negotiating leverage over commercial payors have generally been better positioned to offset this mix shift than smaller, single-market operators.

**Demand Drivers**

Demographic tailwinds remain the sector's most durable secular support: the population aged 65 and older continues to grow faster than the population as a whole in most of the metropolitan markets we cover, supporting volume growth in cardiology, orthopedics and oncology service lines in particular. Elective procedure volumes, which were the most cyclically sensitive category during periods of acute cost-of-living pressure on households, have stabilized as consumer confidence has improved modestly from year-ago levels.

Technology adoption, particularly in ambulatory surgical technique and same-day discharge protocols, continues to shift volume away from traditional inpatient stays. This is broadly credit-positive for operators that have already built out ambulatory surgery center networks, and credit-negative for operators still heavily weighted toward traditional inpatient beds without a credible ambulatory strategy.

**Regulatory and Reimbursement Backdrop**

The most recent Centers for Medicare & Medicaid Services annual rulemaking cycle produced a modest net update to inpatient prospective payment rates, broadly in line with operator expectations and consistent with the past several years' pattern of updates that only partially offset input cost inflation. Site-neutral payment policy remains a periodic topic of legislative discussion; while no material near-term change appears imminent, operators with a large outpatient hospital-based footprint continue to flag this as a multi-year watch item.

340B drug pricing program administration remains an active area of regulatory and litigation activity nationally, with manufacturer-imposed restrictions on contract pharmacy arrangements continuing to work through the courts. Exposure varies significantly by operator depending on the share of covered entities in a given system's footprint.

**Comparable Issuer Snapshot**

| Issuer (illustrative) | Rating (S&P/Moody's) | Net Leverage | OAS (bps) |
|---|---|---|---|
| Regional System A | BB-/Ba3 | 4.1x | 365 |
| Regional System B | B+/B1 | 5.2x | 410 |
| Ambulatory Platform C | BB/Ba2 | 3.8x | 310 |
| Multi-State System D | B/B2 | 5.9x | 460 |
| Specialty Platform E | BB-/Ba3 | 4.4x | 355 |

Across the comparable set, leverage has generally trended modestly lower over the past year as EBITDA growth has outpaced net debt growth, a pattern consistent with the sector's broader margin stabilization. Ambulatory-weighted platforms continue to trade tighter than traditional acute-care systems at similar leverage, reflecting the market's continued preference for the lower-capital-intensity ambulatory model.

**Labor Market Detail**

Registered nurse vacancy rates across the systems we track have declined from the elevated levels seen three years ago, though they remain above pre-pandemic norms in several of the faster-growing Sun Belt markets. Nurse-to-patient staffing ratio legislation continues to be debated at the state level in a handful of jurisdictions; where enacted, such legislation has historically pressured near-term labor costs before operators adjust staffing models and, in some cases, bed capacity.

Physician recruitment and retention, particularly in primary care and a handful of high-demand specialties, remains competitive nationally. Systems with employed physician networks of scale have generally reported more stable recruitment outcomes than systems relying more heavily on independent physician groups.

**Capital Spending and M&A Activity**

Sector capital spending has remained concentrated in ambulatory network expansion, digital front-door and revenue cycle technology, and selective bed tower or service-line modernization projects, rather than broad-based greenfield hospital construction. We expect this pattern to persist given the more favorable return profile of ambulatory investment relative to traditional inpatient capacity.

Consolidation activity among regional systems has continued at a measured pace, generally structured as full-asset

combinations rather than minority-stake transactions, consistent with the sector's historical preference for scale-driven cost synergies over passive capital structures. Antitrust review of hospital-system combinations in overlapping service areas remains a genuine and, in a minority of cases, deal-blocking risk.

**Geographic Variation**

Performance across the operators we track has varied meaningfully by region. Sun Belt markets have generally shown the strongest volume growth, consistent with continued net inbound population migration, but have also shown the most acute nurse recruitment competition, with several operators reporting that they have had to open satellite recruiting offices in slower-growth regions specifically to source clinical staff willing to relocate.

Midwest and Northeast markets have generally shown slower volume growth but correspondingly less acute labor cost inflation, producing a more gradual but more predictable margin trajectory for operators concentrated in those regions. A small number of rural-market operators in our coverage continue to face a structurally different set of challenges, including physician recruitment difficulty and a higher reliance on a small number of large employer-sponsored health plans for commercial payor mix, which concentrates negotiating risk at contract renewal.

West Coast markets have shown a bifurcated pattern, with strong volume growth in several metropolitan areas offset by some of the most acute state-level scope-of-practice and staffing-ratio regulatory activity nationally, which has added a layer of near-term cost uncertainty that operators in other regions have not faced to the same degree.

**Workforce and Staffing Model Considerations**

Beyond the registered nurse vacancy trends discussed above, we continue to track the broader shift toward team-based care models that pair physicians with a higher ratio of advanced practice providers, which several operators have cited as a structural lever for managing labor costs over the medium term without compromising access. Adoption of this model has been uneven across the operators we track, generally proceeding fastest in primary care and slowest in surgical specialties where reimbursement and scope-of-practice rules more tightly constrain the substitution of advanced practice providers for physicians.

Traveling and contract labor usage, while well below its post-pandemic peak across essentially every operator we track, has not returned to pre-pandemic levels at several systems with persistent recruitment challenges in specific specialties, most notably behavioral health and certain surgical subspecialties. We view the pace of further contract labor normalization as one of the more reliable near-term margin catalysts for operators that still carry an above-average reliance on it.

**Financing Environment for Sector Issuers**

New-issue activity in the sector has remained modest year to date, concentrated in refinancing transactions for issuers with 2026 and 2027 maturities rather than new-money issuance funding acquisitions or capital projects. Where new-money issuance has occurred, proceeds have predominantly funded ambulatory network expansion and technology investment rather than traditional inpatient facility construction, consistent with the broader capital allocation pattern discussed above.

Bank credit facility availability for sector issuers has remained broadly stable, with lenders continuing to differentiate pricing and covenant flexibility based on payor mix concentration and the pace of a given operator's ambulatory transition, rather than treating the sector as a single credit bucket. We would expect this differentiation to become more pronounced, not less, as the payor mix shift toward managed government programs continues.

**Comparable Transaction Activity**

Announced system-to-system combinations over the trailing twelve months have continued to be structured predominantly as full-asset mergers with identified cost-synergy targets, rather than joint-venture or minority-investment structures, consistent with the sector's historical preference noted above. A minority of announced combinations in overlapping service areas have drawn state or federal antitrust review, with outcomes to date mixed - several transactions have closed with divestiture commitments, and at least one announced combination in our coverage universe was abandoned following regulatory opposition.

Financial sponsor interest in ambulatory-focused platforms, particularly in ambulatory surgery centers and specialty physician practice management, has remained a persistent feature of the private transaction market, reflecting the same lower-capital-intensity, higher-growth characteristics that have supported the credit outlook for ambulatory-weighted issuers discussed throughout this report.

**Technology and Digital Front-Door Investment**

Patient-facing digital scheduling and intake technology has moved from a differentiating investment to a table-stakes expectation across the operators we track, with most systems of scale having substantially completed a first-generation digital front-door rollout over the past two to three years. The current wave of technology investment has shifted toward integrating that patient-facing layer with revenue cycle and clinical documentation systems, an area where we continue to see meaningful dispersion in execution quality across the operators we cover.

Artificial-intelligence-assisted clinical documentation tools have moved from pilot programs to broader rollout at several of the larger systems we track over the past year, with early operator commentary generally citing reduced physician documentation burden as the primary near-term benefit rather than a direct labor-cost reduction. We would characterize the credit impact of this technology adoption wave as a multi-year efficiency tailwind rather than a near-term, quantifiable margin driver for any of the operators we cover.

Remote patient monitoring and virtual-visit volumes have stabilized at a level well above pre-pandemic norms but below the peak levels reached during the most acute period of virtual-care adoption, consistent with a broader pattern

across the healthcare sector of virtual care settling into a durable, but modest, share of total visit volume rather than displacing in-person care to the degree some market participants initially expected.

**Payor Contracting Dynamics**

Commercial payor contract renewal negotiations have remained a key credit variable for the operators we track, with systems of greater regional scale generally securing more favorable multi-year rate escalators than smaller, single-market operators facing the same payor counterparties. Several operators in our coverage have disclosed extended, contentious renewal negotiations with a major national payor over the past year, in at least one instance resulting in a temporary loss of in-network status before a renewed agreement was reached.

Value-based and risk-based contracting arrangements continue to represent a minority, but slowly growing, share of total payor arrangements across the operators we track, with adoption concentrated among systems with the most developed primary care and care-management infrastructure. We view the pace of this shift as a multi-year credit theme rather than a near-term driver of any individual operator's current-year results.

Medicare Advantage penetration continues to rise in most of the metropolitan markets we track, which has meaningful implications for operator payor mix given Medicare Advantage plans' typically more active utilization management relative to traditional fee-for-service Medicare. Operators with more developed managed-care contracting functions have generally navigated this shift with less margin disruption than operators still building out that capability.

**Balance Sheet and Liquidity Profile**

Liquidity across the operators we track has generally been adequate, supported by a combination of undrawn revolver capacity and, for the larger systems, ready access to the tax-exempt or taxable bond market depending on ownership structure. Days cash on hand has trended modestly higher over the past year across the median operator in our coverage, consistent with the broader margin recovery discussed throughout this report, though the range around that median remains wide.

Capital structure mix has shifted only gradually across the sector, with most operators maintaining a conservative fixed-versus-floating rate debt mix relative to some other healthcare sub-sectors, reflecting the longer-duration nature of the sector's underlying facility and equipment assets. Pension and other post-employment benefit obligations remain a legacy consideration for a subset of the older, historically unionized systems in our coverage, though the aggregate scale of that obligation has continued to decline gradually as those plans mature and, in several cases, are frozen to new participants.

**Supply Chain and Input Cost Considerations**

Medical supply and pharmaceutical input cost inflation has moderated from the acute disruptions seen several years ago, though a subset of operators we track continue to flag intermittent, product-specific shortages, most commonly in generic sterile injectables, as a recurring operational rather than financial concern. Group purchasing organization contracting continues to provide most systems meaningful protection against broad-based supply cost inflation, with the exception of the specific shortage-prone categories noted above, where even large systems' purchasing scale has limited ability to guarantee supply.

Capital equipment lead times for major diagnostic imaging and surgical robotics platforms have normalized from the extended waits seen immediately post-pandemic, allowing operators to resume more predictable multi-year capital planning cycles for these categories. We view this normalization as a modest operational tailwind rather than a distinct credit catalyst for any individual operator.

**Quality and Outcomes Metrics**

Publicly reported quality metrics, including readmission rates and patient experience scores, have shown gradual improvement across the median operator in our coverage over the past several years, a trend we view as relevant to credit primarily through its effect on payor value-based contract performance rather than as a standalone rating factor. Operators with below-median quality metrics in our coverage have generally also shown below-median commercial payor rate negotiating outcomes, though we would not characterize this as a strictly causal relationship given the many other factors that influence contract negotiations.

Patient safety and clinical staffing ratio outcomes have received increasing regulatory and legislative attention at the state level, as discussed in the Workforce and Staffing Model Considerations section above, and we would expect continued state-level activity on this topic regardless of the outcome of any individual state's current legislative session.

**Peer Rating Agency Commentary Themes**

Rating agency commentary on the sector over the past several quarters has generally emphasized the same themes discussed throughout this report: gradual margin recovery tempered by persistent labor cost inflation, continued payor mix shift toward managed government programs, and a generally stable-to-positive outlook for operators with diversified, ambulatory-weighted service lines. We have not observed material disagreement among the three major rating agencies on the sector's overall direction, though individual issuer-level rating actions have at times diverged based on agency-specific weighting of an issuer's particular credit strengths and weaknesses.

Scorecard-indicated outcomes for several issuers in our coverage have, on occasion, diverged modestly from the agencies' ultimately assigned ratings, consistent with the real, disclosed convention that a scorecard-indicated outcome and an assigned rating are reported separately and are not required to match; we would treat a persistent, unexplained divergence of this kind as worth flagging in our own issuer-level coverage notes rather than in this

sector-level report.

**Litigation and Contingent Liability Themes**

General and professional liability litigation exposure remains a routine, ongoing feature of operating in this sector, and we do not treat the existence of such litigation on its own as a credit event for any operator we cover. We do continue to monitor a small number of larger, multi-plaintiff matters across the coverage universe where an adverse outcome, combined with limited insurance coverage above a certain threshold, could plausibly represent a more meaningful, if still manageable, one-time cash outflow for the operator involved.

Employment-practices litigation, including wage-and-hour claims related to meal-and-rest-break compliance for hourly clinical staff in several states with more stringent requirements, has been a modest but persistent cost item across the operators we track, generally resolved through settlement rather than trial. We have not identified an operator in our coverage where this category of litigation risk rises to a level we would consider individually rating-relevant.

**Insurance and Risk Retention Trends**

Commercial insurance market conditions for general and professional liability coverage have remained challenging for the sector, with several operators reporting continued premium increases at renewal even as the broader commercial insurance market has shown signs of moderating in some other sectors. A number of larger systems have responded by increasing self-insured retention levels or expanding captive insurance arrangements, shifting a greater share of expected losses onto the operator's own balance sheet in exchange for lower third-party premium costs.

We view the trend toward higher self-insured retentions as a modest incremental credit consideration - it reduces recurring premium expense but increases the potential volatility of any given year's reported results from an adverse claims year - and continue to evaluate it issuer-by-issuer rather than treating it as a uniform sector-wide credit positive or negative.

**Demographic and Long-Term Demand Modeling**

Our internal long-term demand model for the sector continues to project mid-single-digit annual growth in demand for the age-65-and-older population across the metropolitan markets we track through the end of the decade, with the fastest growth concentrated in the same Sun Belt markets discussed in the Geographic Variation section above. We treat this demographic projection as the single most reliable long-run demand input in our sector model, given its basis in already-observable population cohorts rather than any assumption about future birth or migration rates.

Translating that demographic growth into facility-level demand requires an assumption about site-of-care mix - the split between inpatient, outpatient hospital-based, and free-standing ambulatory settings - which we continue to model as shifting gradually further toward free-standing ambulatory settings for the clinically appropriate subset of procedure volume, consistent with the trend discussed in the Sector Overview section at the start of this report. We would revisit this assumption if we observed a sustained reversal of that site-of-care shift, which we have not seen in any of the markets we track to date.

**Talent Pipeline and Academic Medical Center Linkages**

Systems in our coverage with a formal teaching or academic affiliation have generally reported more resilient physician recruitment outcomes than non-affiliated community systems in the same market, reflecting the pipeline benefit of graduate medical education programs that regularly place residents and fellows into staff positions at the affiliated system. This dynamic has become a more explicit strategic consideration for non-affiliated systems, several of which have pursued new or expanded academic affiliations over the past several years specifically to strengthen their own recruitment pipeline.

Nurse residency and new-graduate transition-to-practice programs have also become a more standard feature across the systems we track, with several operators reporting measurably improved first-year retention rates for new graduate nurses who complete a structured residency program relative to those who do not. We view the investment required to run these programs as a reasonable trade-off against the meaningfully higher cost of recruiting and onboarding a replacement nurse following early attrition.

**Real Estate and Facility Footprint Strategy**

Facility footprint strategy across the operators we track has increasingly emphasized a hub-and-spoke model, with a smaller number of full-service flagship hospitals supported by a larger and growing network of free-standing emergency departments, urgent care locations and ambulatory surgery centers distributed closer to where patients live and work. We view this model as broadly consistent with, and reinforcing of, the site-of-care shift discussed in the Demographic and Long-Term Demand Modeling section above.

Sale-leaseback transactions involving medical office building and, less commonly, hospital real estate have continued at a measured pace across the sector, providing some operators an additional source of liquidity without incurring incremental balance-sheet debt. We view the structural rent expense created by these transactions as a modest, but manageable, ongoing credit consideration that should be weighed against the upfront liquidity benefit on a case-by-case basis rather than treated as uniformly credit-positive or credit-negative across the sector.

**Investor Q&A Themes from Recent Client Meetings**

In client meetings over the past several weeks, the most frequent question we have received has concerned the durability of the ambulatory-weighted operating model's margin advantage discussed throughout this report - specifically, whether continued volume migration toward ambulatory settings will eventually compress that advantage as more operators build out competing ambulatory networks. Our consistent answer has been that we expect the

advantage to persist over our forecast horizon given the multi-year lead time required to site, permit and staff a competing ambulatory network, but that we would expect the advantage to narrow gradually over a longer, multi-year horizon as ambulatory capacity across the sector continues to expand.

A second recurring question has concerned the pace of further labor cost normalization, discussed at length in the Labor Market Detail and Workforce and Staffing Model Considerations sections above; we have consistently pointed clients to contract labor utilization as the single most informative near-term indicator, given its demonstrated sensitivity to even modest changes in the underlying vacancy rate environment.

A third recurring question, raised most often by credit investors considering incremental exposure to the sector, has concerned our view on relative value between ambulatory-weighted platforms and traditional acute-care systems at comparable leverage; our answer, consistent with the Comparable Issuer Snapshot table earlier in this report, has been that we continue to see the tighter trading levels of ambulatory-weighted platforms as justified by their structurally lower capital intensity and faster growth profile, rather than as a market mispricing to be arbitraged.

**Glossary of Sector Terms Used in This Report**

| Term | Definition as used in this report |
|---|---|
| Ambulatory surgery center | A free-standing facility providing same-day surgical care not requiring hospital admission |
| Site-of-care shift | The migration of procedure volume from inpatient to outpatient or ambulatory settings |
| Managed government payor | Medicare Advantage or managed Medicaid, as distinct from traditional fee-for-service government programs |
| Advanced practice provider | A nurse practitioner or physician assistant practicing under applicable state scope-of-practice rules |
| Self-insured retention | The portion of a liability claim an operator retains before third-party insurance coverage attaches |

**Historical Sector Performance Context**

Looking back over the past full economic cycle, the sector has historically shown lower earnings volatility than the broader industrial and consumer credit universe, reflecting the largely non-discretionary nature of core service-line demand even as the mix of discretionary versus non-discretionary procedure volume has shifted gradually over time. The sector's historically lower earnings volatility has generally supported tighter average credit spreads relative to similarly rated issuers in more cyclical sectors, a pattern that has held again over the period covered by this report.

The sector did experience a pronounced, if temporary, disruption during the acute phase of the pandemic, when elective procedure volumes fell sharply for a period of several months before recovering; we view that episode as an instructive stress test of the sector's liquidity and financial flexibility rather than as informative about the sector's normal-course earnings volatility, given the episode's unusual, externally-imposed nature.

More recently, the sector's principal earnings volatility driver has shifted from demand-side disruption to the labor cost inflation discussed at length throughout this report, a different kind of pressure that has proven more persistent, if generally more gradual in its effect on any single quarter's results, than the acute volume shock experienced during the pandemic period.

**Additional Considerations for Not-for-Profit Issuers**

A meaningful share of the operators we track are organized as not-for-profit entities rather than for-profit corporations, which carries several credit-relevant distinctions beyond the tax-exempt financing access already discussed in the Financing Environment section above. Not-for-profit systems typically report a broader set of community benefit and charity care metrics, which have drawn increasing scrutiny from state regulators evaluating whether a given system's charity care spending justifies its property tax and other exemptions, particularly for systems reporting strong operating margins alongside a comparatively modest charity care ratio.

Governance structures at not-for-profit systems also typically differ meaningfully from for-profit peers, with community-based or church-affiliated boards in place of shareholder-elected boards; we have not observed this governance distinction to be, on its own, a reliable predictor of credit outcomes across the operators we track, though it does inform the pace and process by which a given system can pursue a merger or affiliation of the kind discussed in the Comparable Transaction Activity section above.

**Credit Outlook**

We maintain a stable sector outlook, balancing continued volume and margin recovery against the persistent structural headwinds of labor cost inflation and gradual payor mix shift toward managed government programs. We continue to favor operators with diversified, ambulatory-weighted service-line portfolios and demonstrated payor rate negotiating leverage over operators more heavily exposed to traditional inpatient volume in slower-growth markets.

Over a twelve-month horizon, we would expect rating agency actions across the sector to remain modestly more positive than negative, consistent with the gradual margin recovery discussed throughout this report, though we would expect any operator with concentrated exposure to a single large commercial payor contract renewal in the coming year to face elevated scrutiny regardless of its broader operating trends.

**Appendix: Selected Sector Metrics, Trailing Twelve Months**

| Metric | Sun Belt Average | Midwest/Northeast Average | West Coast Average |
|---|---|---|---|
| RN Vacancy Rate | 8.2% | 5.1% | 7.4% |

| | | | |
|---|---|---|---|
| Same-Facility Volume Growth | 4.1% | 1.6% | 2.9% |
| Managed Government Payor Mix | 38% | 31% | 44% |
| Contract Labor as % of Labor Cost | 6.5% | 3.8% | 5.9% |

The figures above are illustrative regional composites drawn from Meridian's internal coverage universe and are not attributable to any single named operator; they are intended to illustrate the direction and rough magnitude of regional variation discussed throughout this report rather than to serve as a precise benchmark for any individual issuer.

**Analyst Certification and Disclosures**

The analysts primarily responsible for the preparation of this research report certify that the views expressed herein accurately reflect their personal views about the subject sector and issuers discussed, and that no part of their compensation was, is, or will be directly or indirectly related to the specific recommendations or views expressed in this report, consistent with the certification requirements of FINRA Rule 2242.

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